the senses,—*e.g.*, the action of A–C on the sensory, and of A–D on the motor tissues, etc. Much clinical experience corroborates these and other qualities ascribed to these diverse combinations, of quantity and tension, and of the primary and secondary currents.

In the beginning of electric practice it is quite possible, as in other methods of treatment, that disappointments may be met with. In every such case it will be well to reconsider the plan, and in so doing, discover the fault,—which will often be found in the scheme or execution of the treatment itself. Frequently the patient may have transgressed, and with the aid of cranial diagnosis, or otherwise, the cause must be ascertained. Consider also whether the manipulations were not of too long duration. Aggravation of original, and even additional, symptoms often proceed from this cause alone. Ten minutes, twenty minutes,—rarely over thirty minutes,—will be the proper time. Fatigue is ominous of a bad effect. The patients must not delude themselves with the idea that a long treatment is proportionately more effectual.

Treatments should be given IN THE MANNER and for the TIME directed. The battery cannot be used carelessly, without great danger.

Excessive strength of the current is to be avoided. Increasing the distance between the electrodes, light pressure, the regulation of the cylinder, control this. Generally the feelings are a good guide,—keeping outside of the limits of actual pain.

Never give treatment over the lungs (back or front) or the front of the head.